ANNUAL TECHNICAL CONSULTATION

DELIVERING THE GLOBAL PROMISE TO END FGM BY 2030

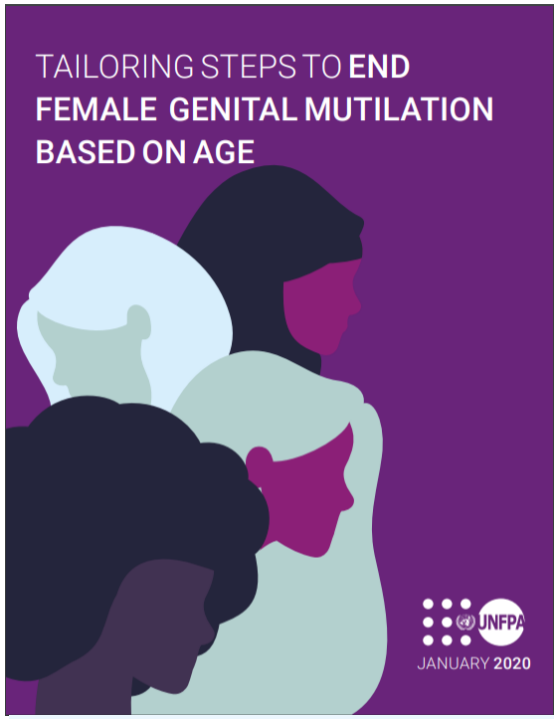
Session 1.1

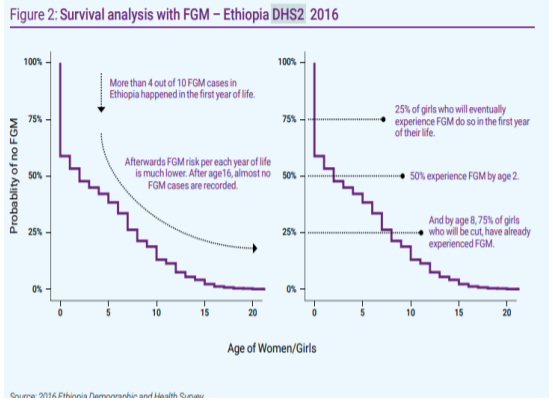
FGM Incidence Risk:

Data, Measurement and Analytical Insights

Romesh Silva, Senior Technical Specialist, Technical Division, UNFPA

6+ years of collaboration b/w statisticians/demographers & FGM experts





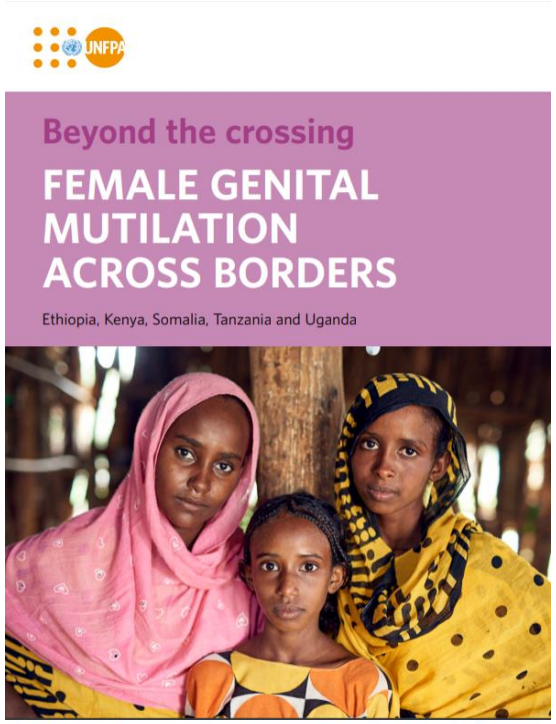
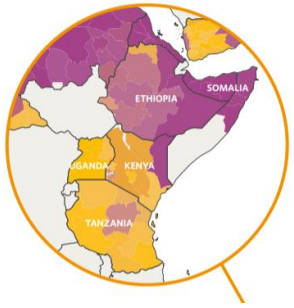


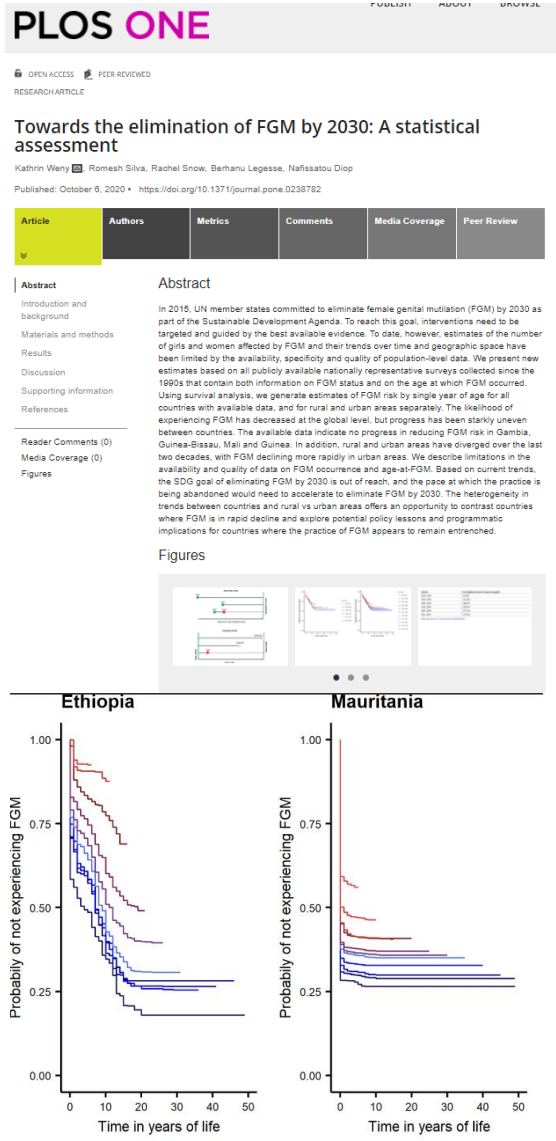
Figure 1. Prevalence of female genital mutilation among women aged 15 to 49, latest available survey

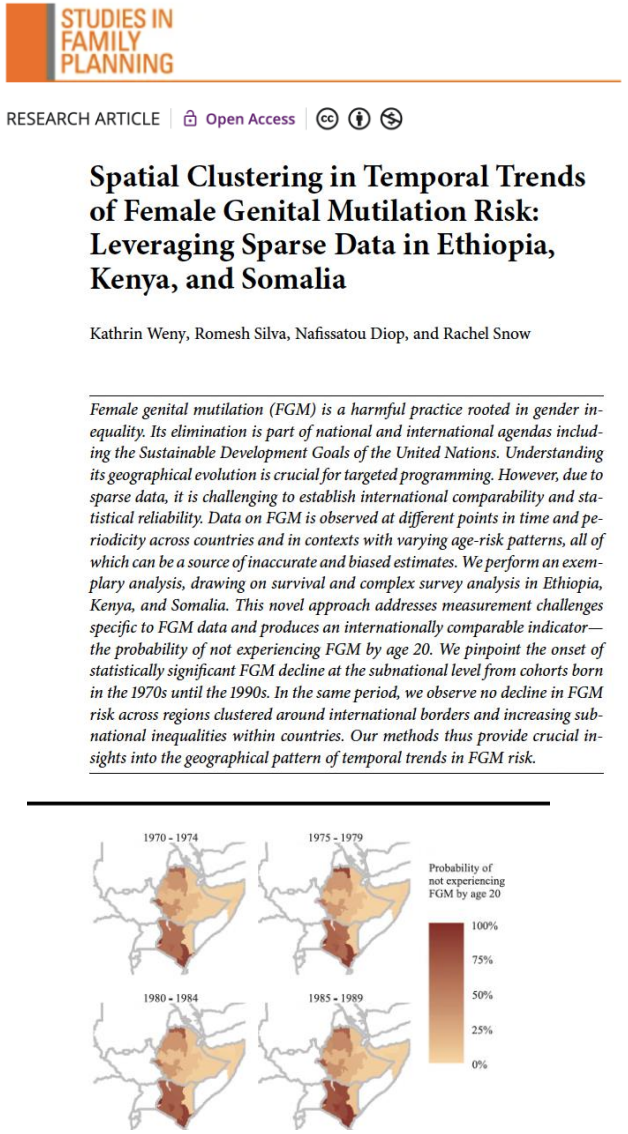


Source: Latest available nationally representative household survey if conducted in the last 10 years. The source of the shape files are:
a) the latest DHS survey (Benin, Burkina Faso, Chad, Cote d'Ivoire, Egypt, Eritrea, Ethiopia, Ghana, Guinea, Kenya, Liberia, Niger, Nigeria, Senegal, Sierra Leone, Tanzania, Gambia, Togo, Uganda, Yemen),
b) the MIS (Mali) or OCHA's data base on administrative boundaries (Guinea-Bissau, Sudan, Iraq, Central African Republic, Mauritania, Somalia-Northeast Zone and Somalia - Somaliland), available at: https://data.humdata.org/

Disclaimer: The designations employed and the presentation of material on this map do not imply the expression of any opinion whatsoever on the part of UNFPA concerning the legal status of any country, territory, city or area or its authorities, or concerning the delimitation of its frontiers or boundaries.

Note: In 2011, a MICS was conducted separately in both Somalia Northeast Zone and Somaliland.





Overview

1. **Context and Motivation:** Why an FGM Incidence Risk Measurement Framework?
2. **Levels and Trends:** What does current and future FGM risk look like?
3. **Methodological Framework:** Applying survival analysis to FGM survey data
4. **Subnational FGM Incidence Risk:** Clustering and increased subnational inequalities
5. **Future data + measurement directions:** Multiple data sources + refined projections

Context & Motivation: Why an FGM Incidence Framework?

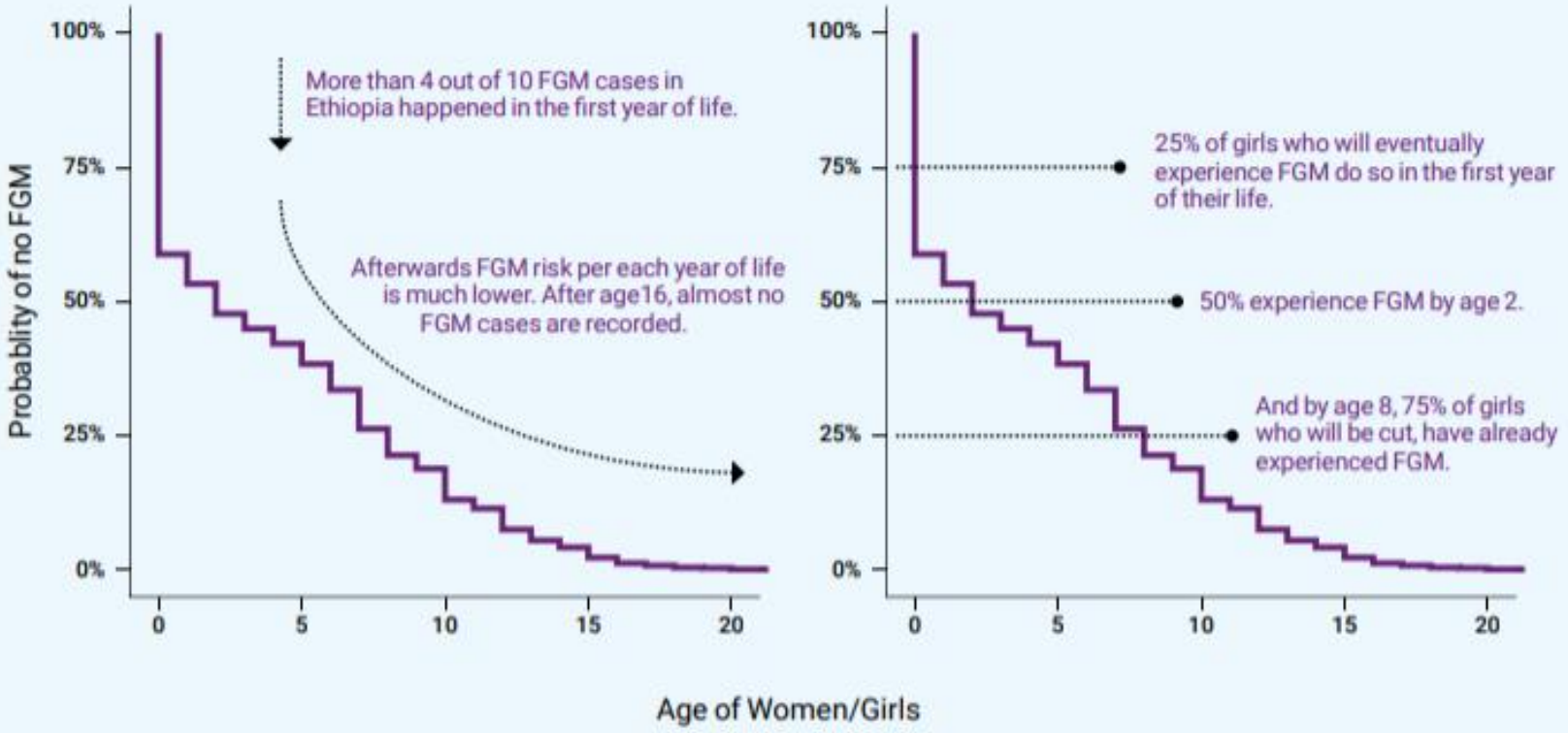
SUSTAINABLE DEVELOPMENT GOALS

Goal: Eliminate **all** acts of FGM by 2030 (i.e. zero FGM incidents by 2030)

5 GENDER EQUALITY

68 million women and girls are at risk of FGM between 2015 and 2030

Figure 2: Survival analysis with FGM – Ethiopia DHS2 2016



Source: 2016 Ethiopia Demographic and Health Survey.

Context & Motivation: Why an FGM Incidence Framework?

Figure 1: Projected number of girls at risk of FGM incidence, 2015-2030

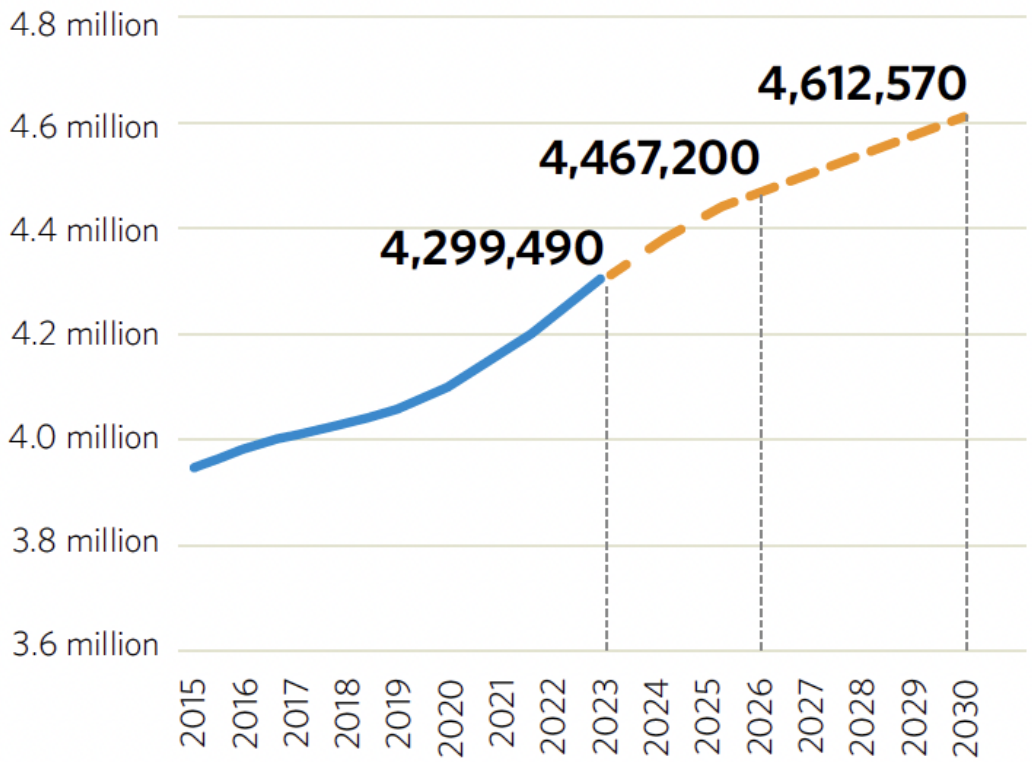
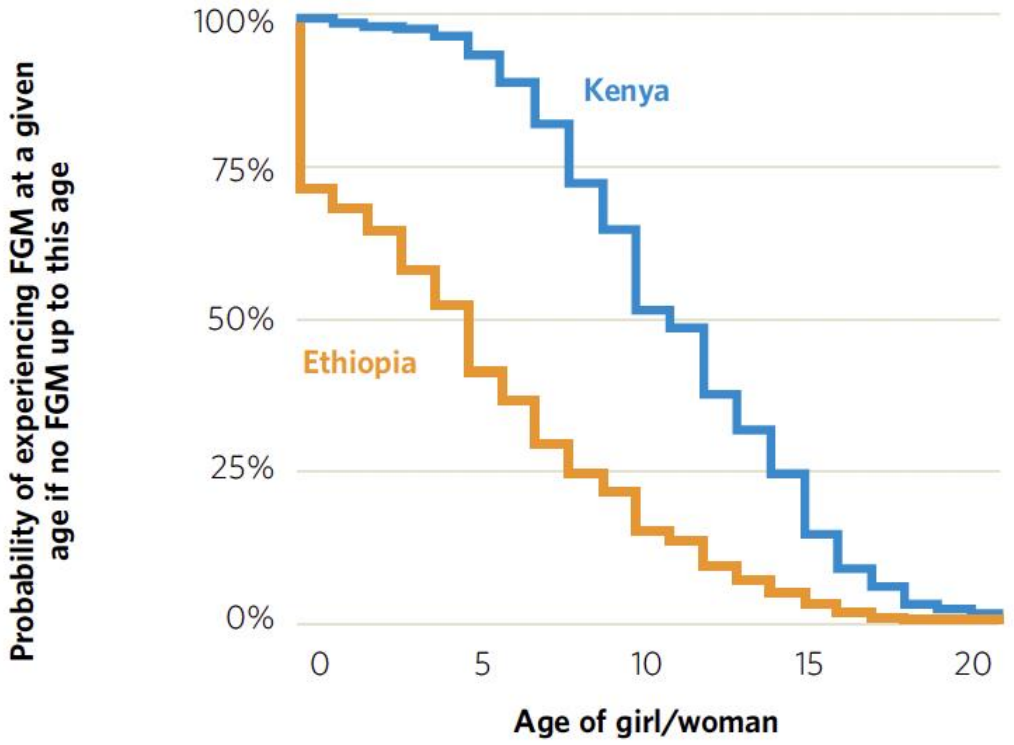


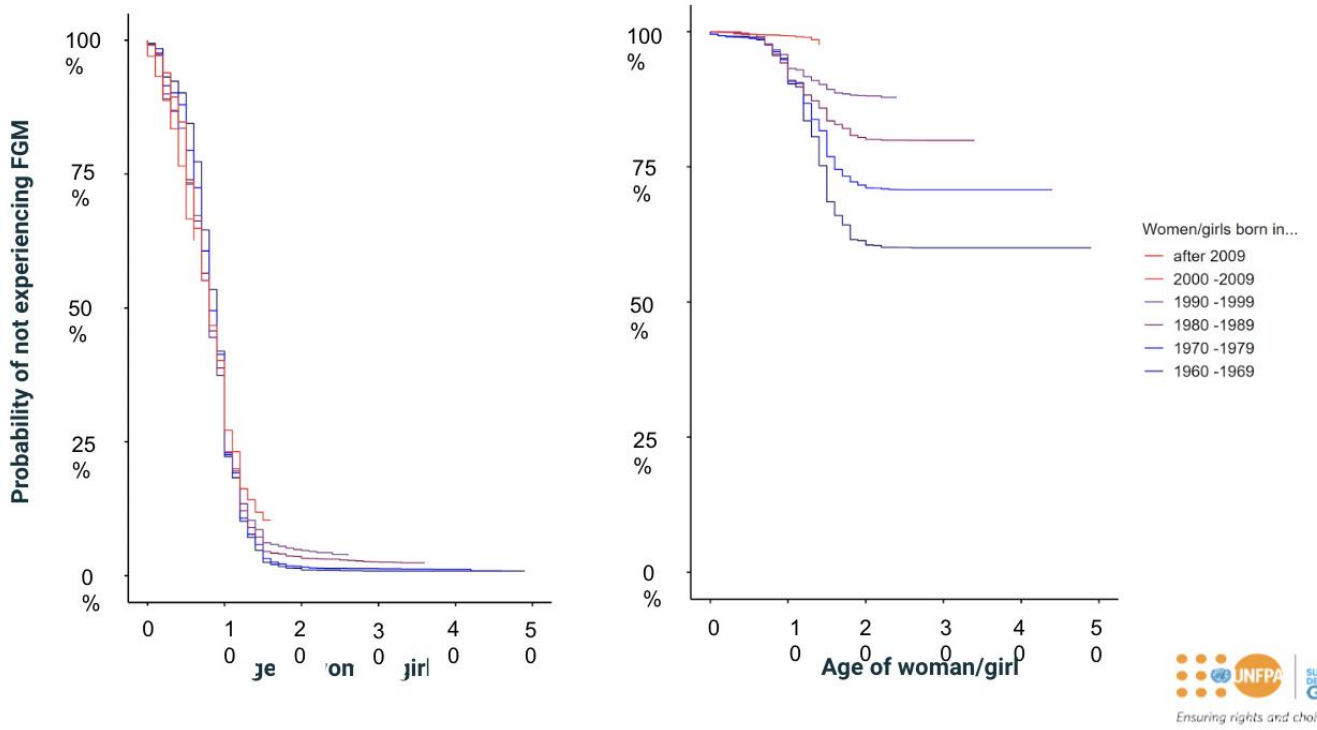
Figure 2: Risk of FGM incidence in Ethiopia and Kenya, by age of girls and women



Levels & Trends: What does current and future FGM risk look like?

Comparison across birth cohorts to estimate trend

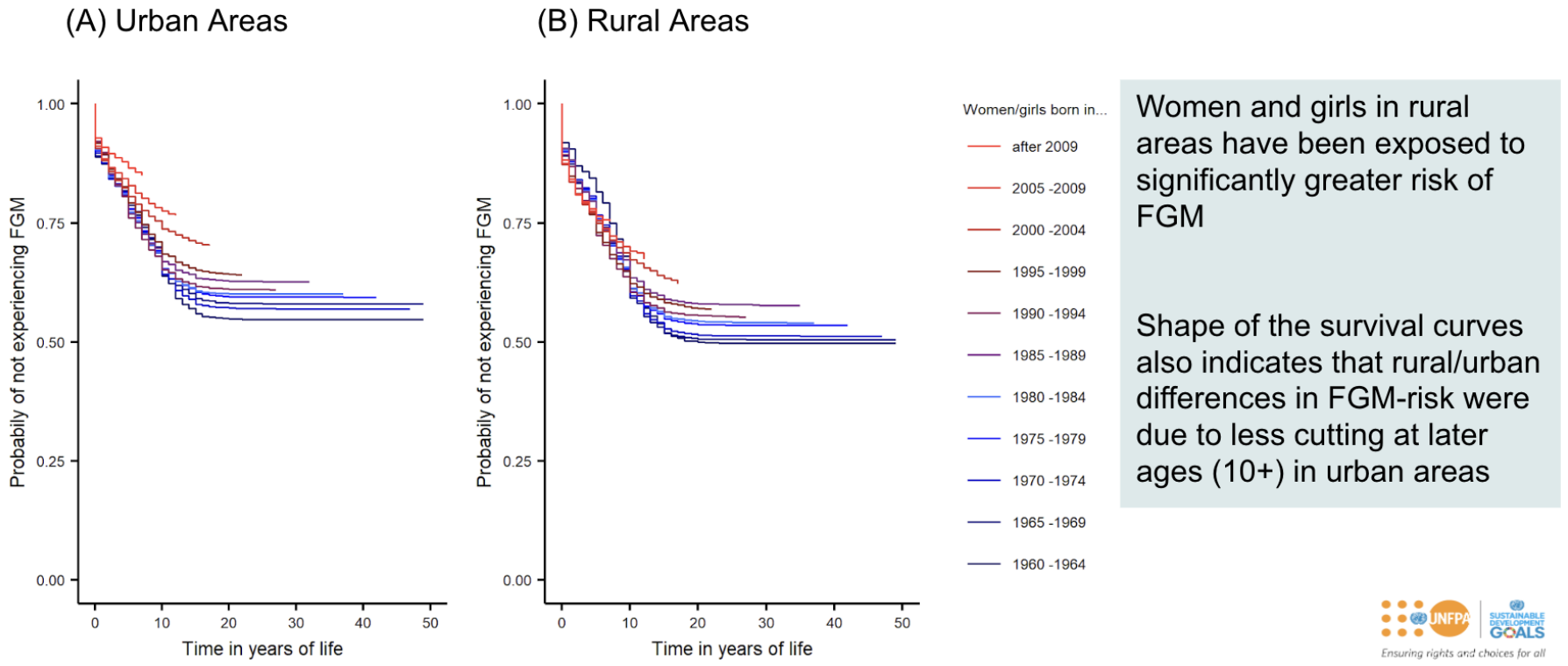
Kaplan Meier estimates (Guinea/Kenya)



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SUSTAINABLE DEVELOPMENT GOALS
Ensuring rights and choices for all

Levels & Trends: What does current and future FGM risk look like?

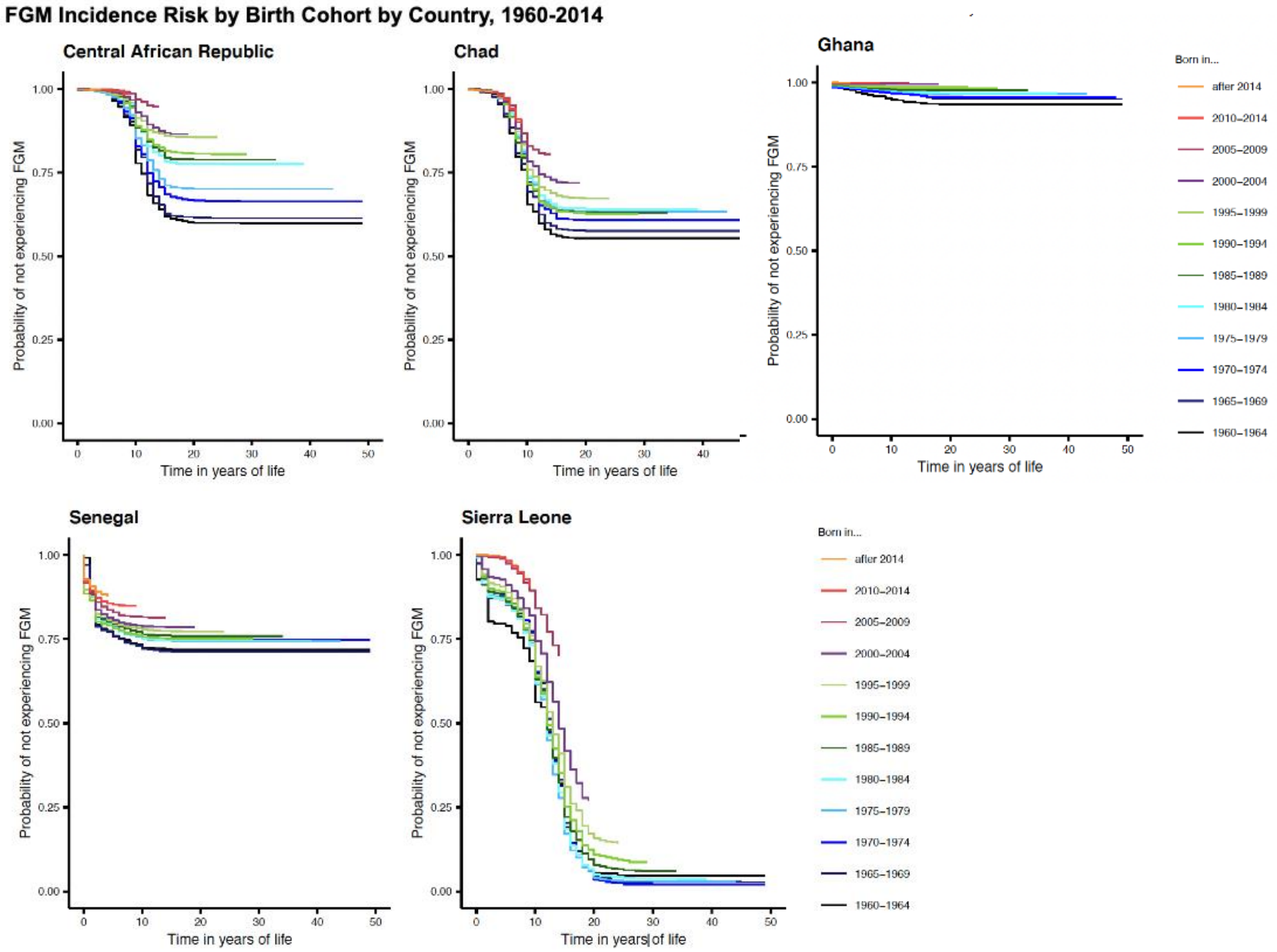
Faster decline in FGM observed in urban areas compared to rural areas



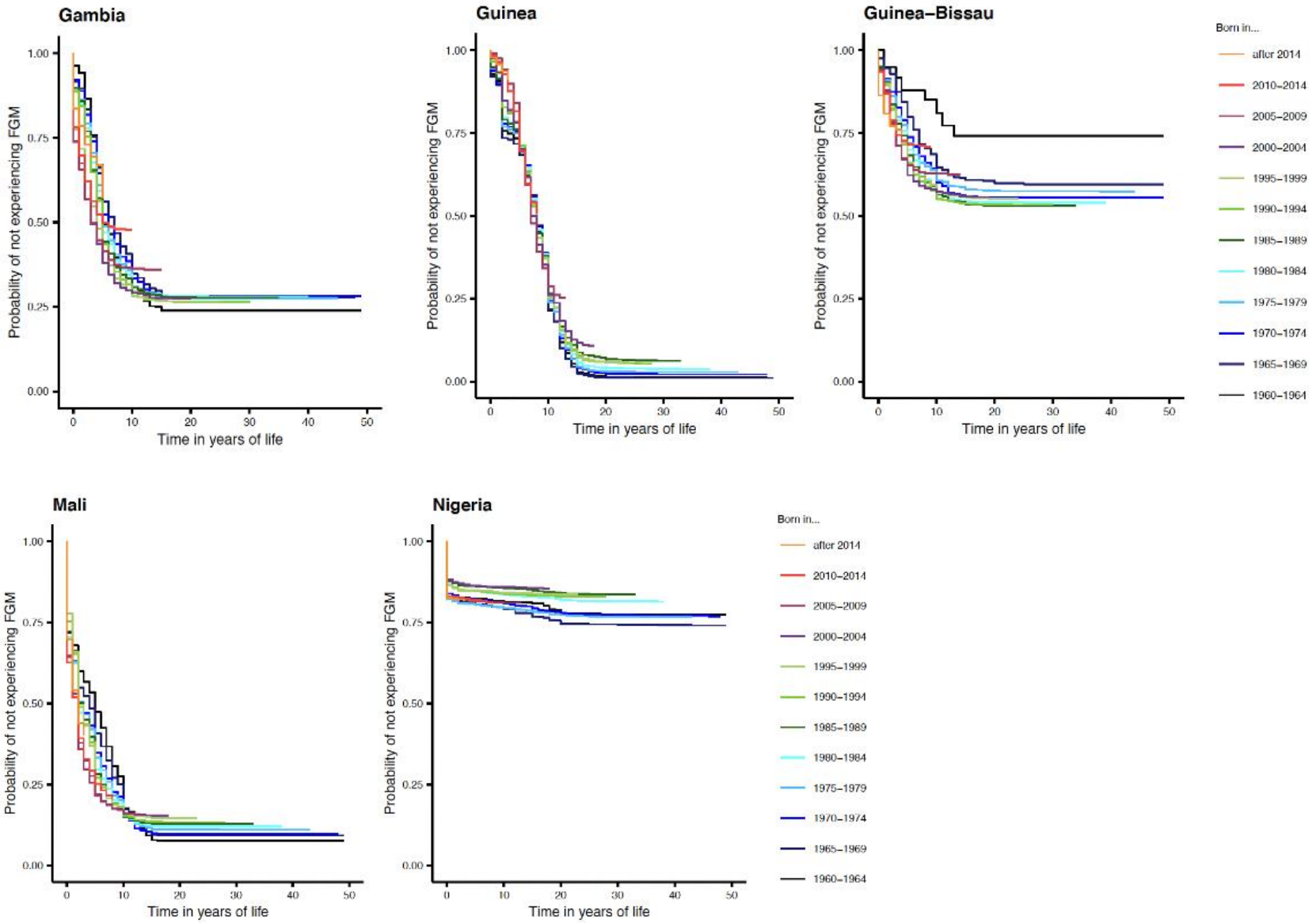
Women and girls in rural areas have been exposed to significantly greater risk of FGM

Shape of the survival curves also indicates that rural/urban differences in FGM-risk were due to less cutting at later ages (10+) in urban areas

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Ensuring rights and choices for all



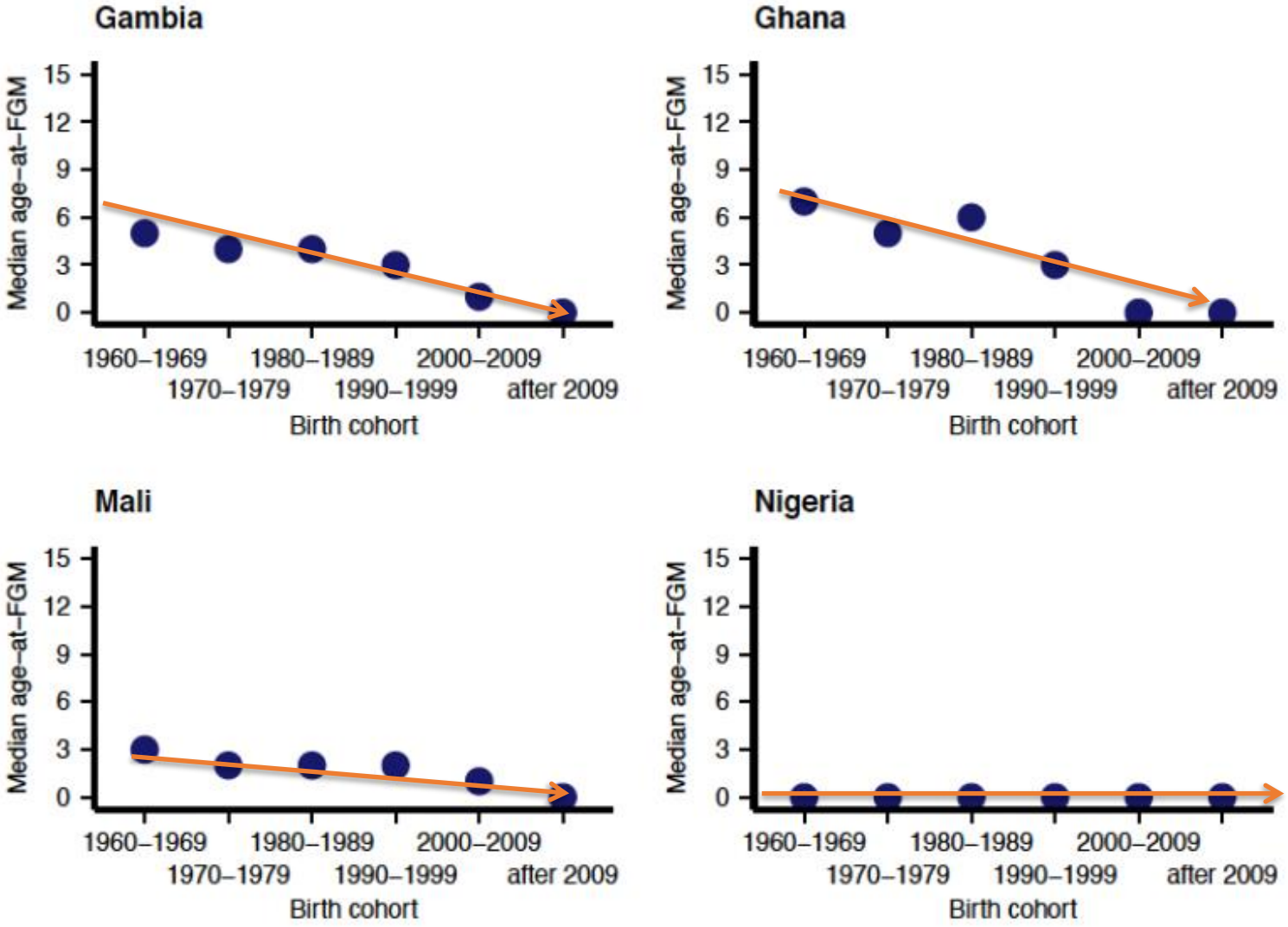
Recent declines in FGM Incidence Risk
Central African Republic
Chad
Ghana
Senegal
Sierra Leone



No measurable reductions in FGM Incidence Risk

- Gambia
- Guinea
- Guinea-Bissau
- Mali
- Nigeria

Levels & Trends: What does current and future FGM risk look like?



Notable variation in age at FGM across birth cohorts in some countries

Methodological Framework:
FGM Incidence Risk Measurement

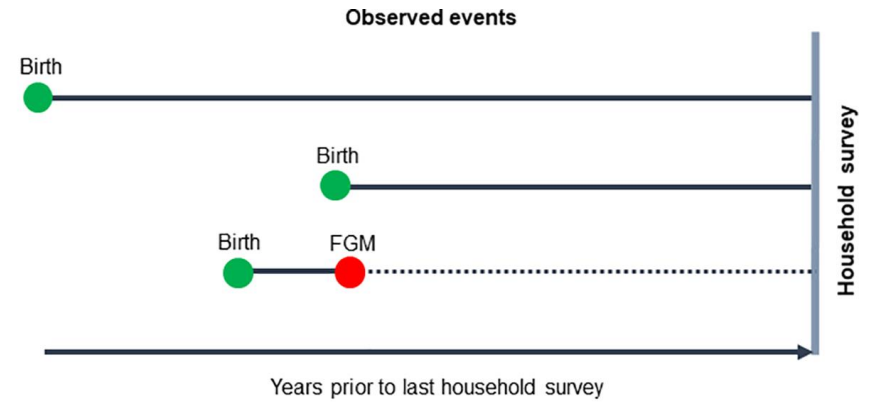
$$\text{Probability of being cut in year } i = \frac{\text{Girls being cut in year } i \text{ of their life}}{\text{Number of girls not having been cut at the start of year } i \text{ and older than } i}$$

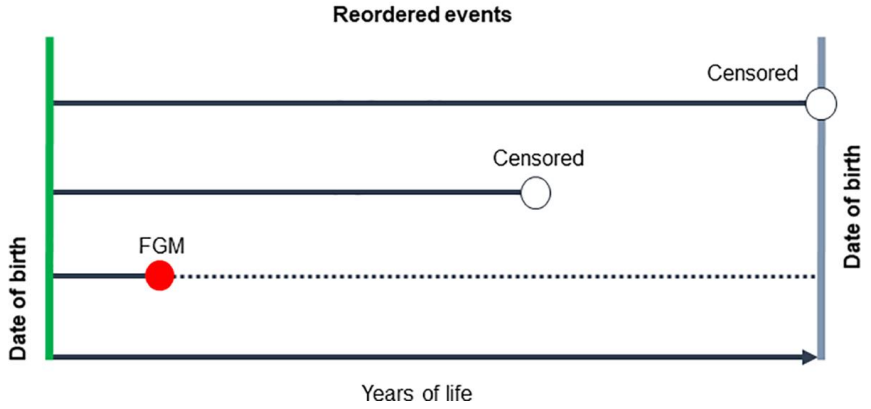
Recently available household survey data

Table 1. Unweighted count of women and girls by country, year, and survey type, DHS and MICS Surveys with FGM Modules conducted 2017-2020

Country	Survey Year	Survey	Number of women/girls
Central African Republic	2018-2019	MICS	18,388
Chad	2019	MICS	48,313
Gambia	2019-2020	DHS	11,070
Ghana	2017-2018	MICS	25,344
Guinea	2018	DHS	19,159
Guinea-Bissau	2018-2019	MICS	19,763
Mali	2018	DHS	10,674
Nigeria	2018	DHS	49,810
Senegal	2018	DHS	17,669
Senegal	2019	DHS	16,399
Sierra Leone	2019	DHS	26,049

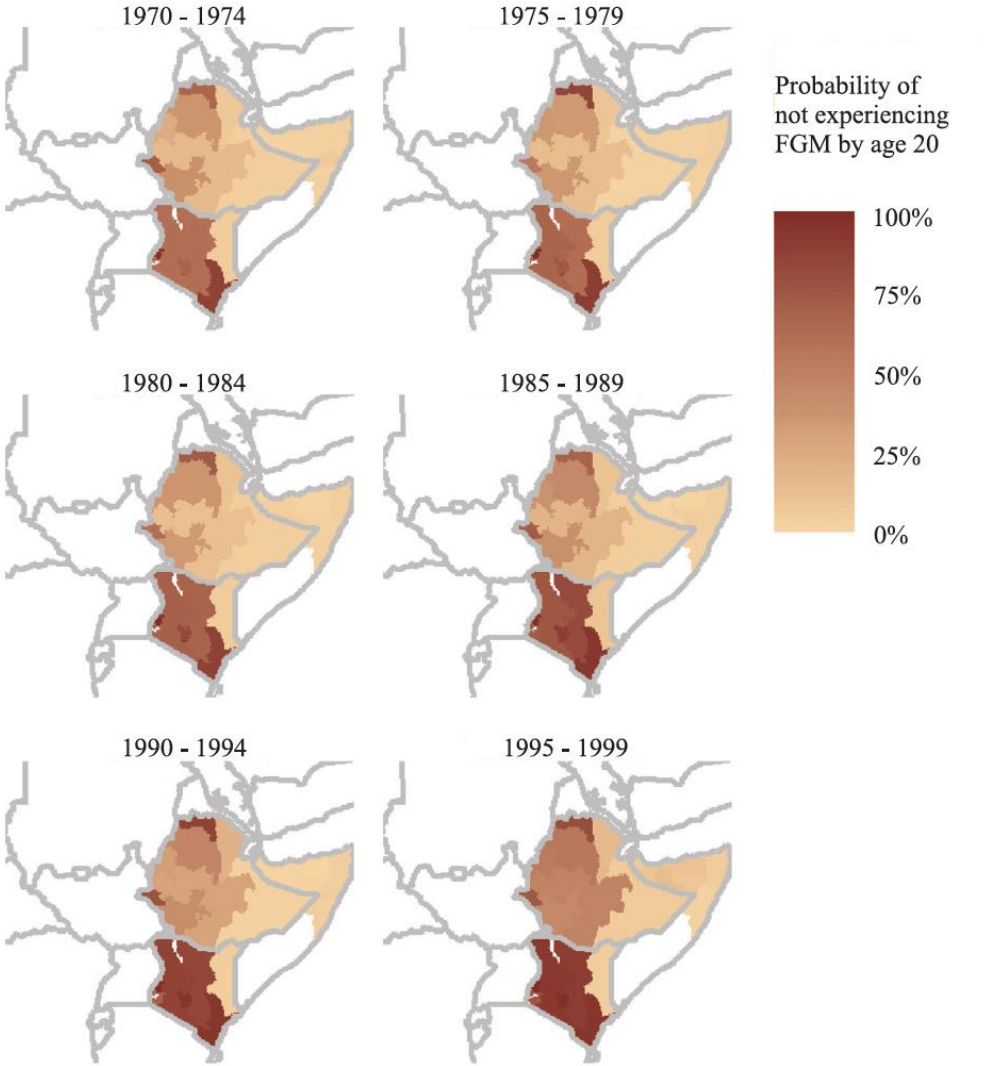
Cohort analysis of time-to-FGM-event





Subnational FGM Incidence Risk

Geographic clustering and increased inequalities within countries



Heightened subnational risk of FGM incidence is heavily concentrated in cross-border areas in Ethiopia, Kenya, Somalia and other countries*

Future Data and Measurement Directions

1. Go beyond DHS/MICS surveys for FGM Incidence Risk Measurement
→ leverage other data sources

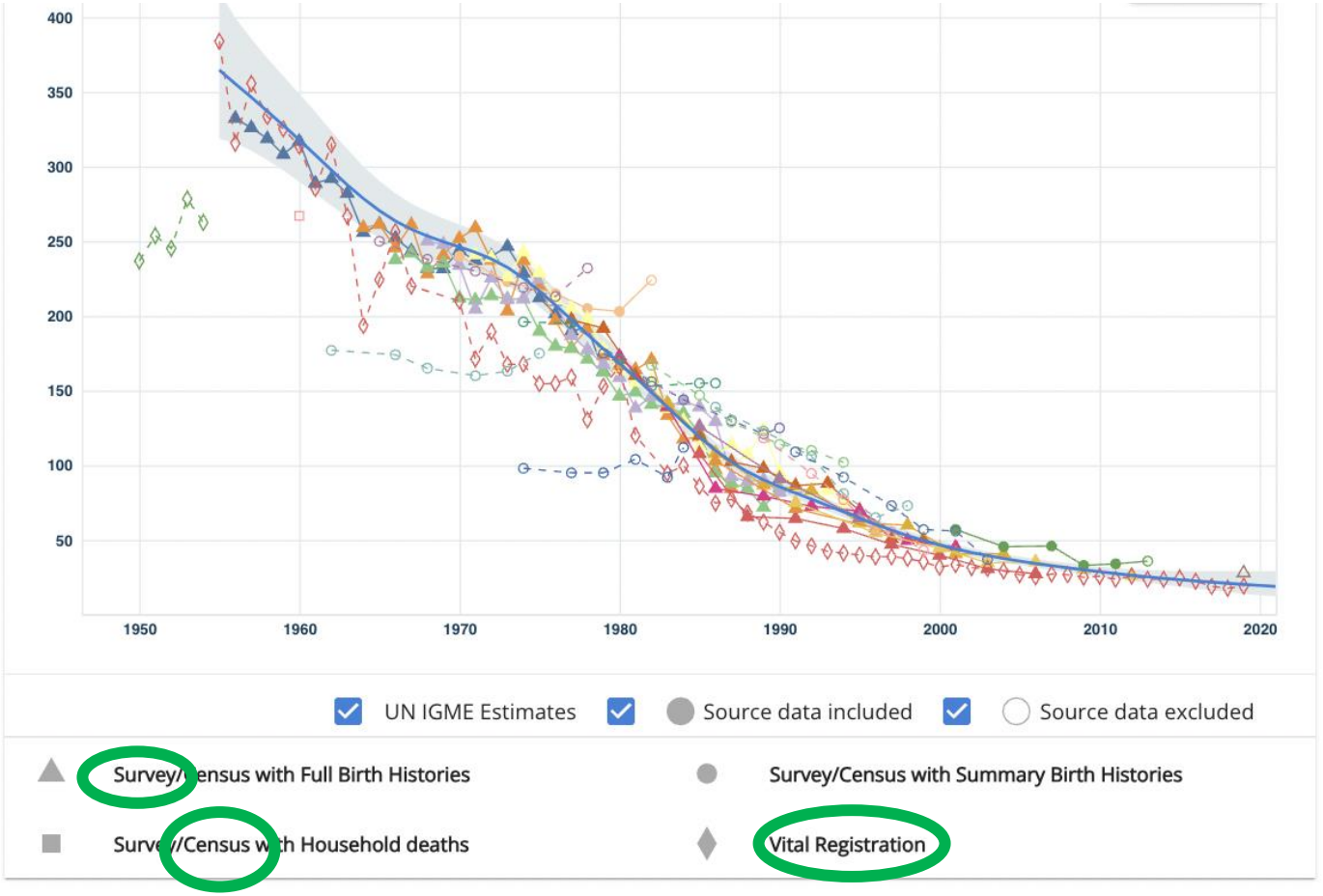
2. When projecting future FGM incidence risk,
(a) model current and future population dynamics; and
(b) incorporate social and behavioral norms changes into future FGM incidence risks

Future Data and Measurement Directions

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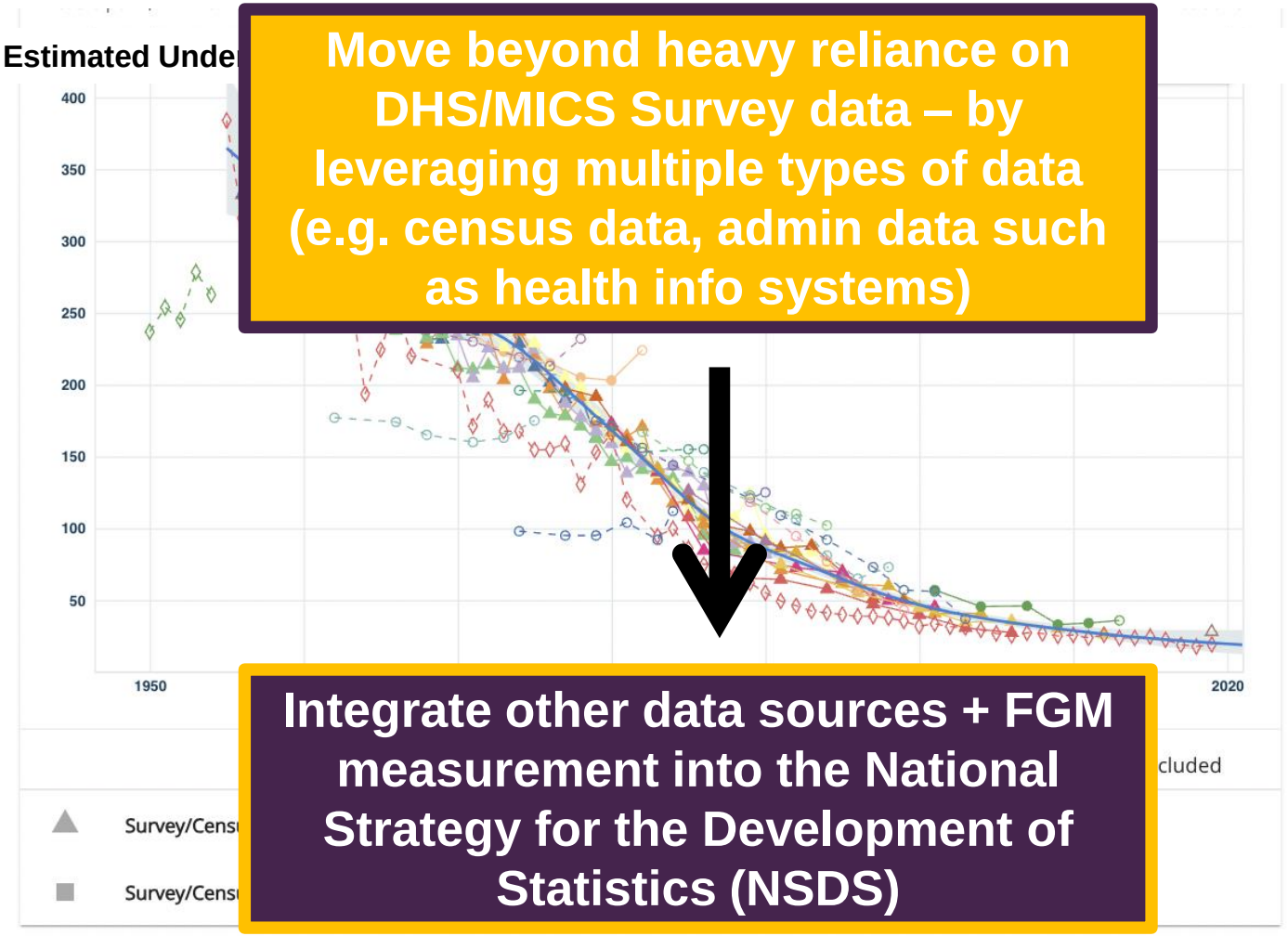
Estimated Under-Five Mortality Rate by Data Source Type, Egypt, 1955-2020



Future Data and Measurement Directions

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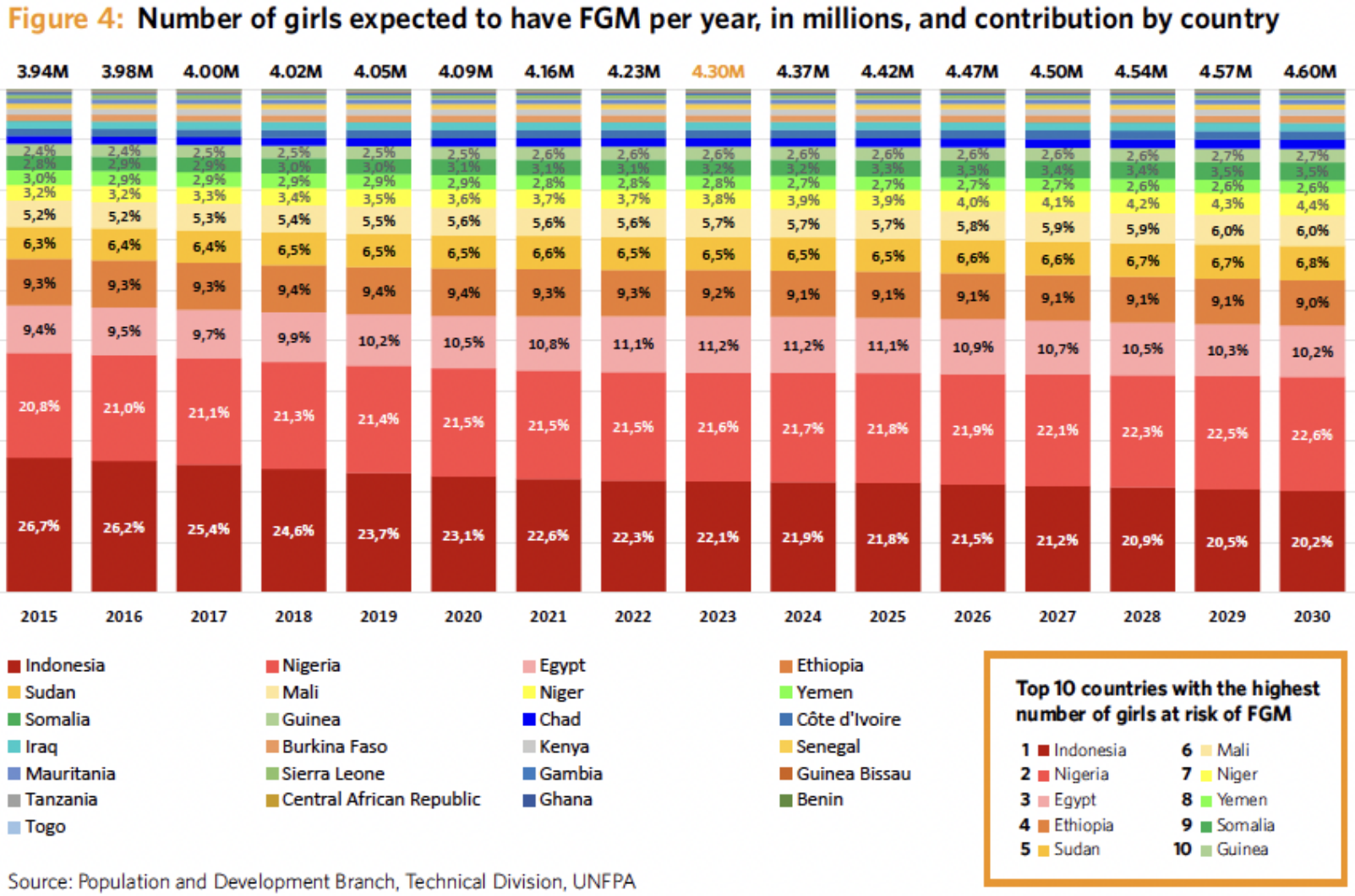
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Move beyond heavy reliance on DHS/MICS Survey data – by leveraging multiple types of data (e.g. census data, admin data such as health info systems)

Integrate other data sources + FGM measurement into the National Strategy for the Development of Statistics (NSDS)

Future Data and Measurement Directions



Potential Future Improvements

Directly model population dynamics and megatrends

Incorporate social/behavioral factors in future modeled scenarios using ACT data etc.